

The Psychotherapies: Benefits and Limitations*

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Benefits and limitations are discussed in five major areas: treatment, training, diagnosis, research, and review. Strides in the standardization of the psychotherapies, development of innovative techniques, and integration of theoretical and clinical models are interfaced with economic concerns and the impact of public policy on the future of the field.

Psychotherapists are faced with solvable questions that are unimportant, and important questions that are unsolvable.

The psychotherapies are greeting the 1980s with unprecedented promise and its constant companion, challenge. On the positive side of the scale are enormous strides in the "science" of psychotherapy, including major progress in the standardization of diagnosis, training, and treatment¹⁻³; improved research tools for establishing therapeutic efficacy^{4,5}; growth and development of new, innovative clinical modalities, especially short-term techniques in keeping with the temper of the times^{6,7}; and a variety of cooperative efforts to combine and integrate different therapeutic approaches drawn from the rich armamentarium of treatments available today.⁸ The latter marks our entry into the latest stage of psychotherapeutic development—rapprochement, a needed and welcome change after years of unbridled expansion and divisiveness in the field.

On a more sociological (and political) plane, out of an "age of ethical crises"⁹ has also come greater attention to accountability, morality, and values in psychotherapy and the establishment of codes of conduct to better protect the rights of therapist and patient.^{10,11} To this may now be added the climate of today's economy, particularly the rising costs of health care, which has meant increasing interest and investigation into the efficacy, safety and efficiency of the psychotherapies.¹² Easier access to new technology will bring increased application of computerized assessment and cost-containment procedures to the mental health field.¹³

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Interfaced with these attempts and accomplishments are, of course, unresolved problems and challenges. Included here are the negative effects from the proliferation of diverse approaches—polarization, conceptual confusion, competition, and identity crises; the threat of impending public policies and political pressures that have economically placed “cost-effectiveness” at the pivot of mental health care; research limitations which, despite major technological progress, have still meant inconclusiveness of results, new differences of opinion and unanswered questions, and the failure of findings to sufficiently impact upon the clinician’s actual work or his most ardent partisan allegiances. In an earlier paper I suggested that we have contributed largely to this unsettled state of affairs by “our questionable clinical practices, theoretic confusion, research inadequacies, political ineptness, and professional ambivalence.”¹⁴

In light of the above, it is understandable that while some observers of the field are as yet viewing the changes in psychotherapy with some combination of nostalgia, conflict, and deep concern,¹⁵⁻¹⁸ others are expressing a sense of renewed optimism and hope for the future.^{8,19-21} As such, the following exploration of the benefits and limitations of the psychotherapies will be addressed across five major (albeit overlapping) areas: *Treatment and Training*, *Diagnosis*, and *Research and Review*.

TREATMENT AND TRAINING

An excessively large number of good ideas in psychotherapy practice are obstructing the development of better ones.

The most potentially beneficial as well as problematic area here has been the proliferation of widely diverse theories, schools, and clinical techniques. According to one recent count, which literally runs the gamut “from A” (Active analytic therapy) “to Z” (Zaraleya psychoenergetic technique), there are more than 250 varieties now competing on the psychotherapeutic scene.²² My own tally has reached over 450 types polled nationwide. The negative consequences and limitations from this phenomenon have included professional rivalry, dogmatism, overchoice, role diffusion, identity confusion, fragmentation, and the like, which all have contributed to a profound sense of “psychiatry in crisis.”¹⁵ However, the field seems to have gone about as far as it can go in terms of the sheer expansion of its boundaries and the polarization of its parts, and it is beginning to reap the benefits of the wealth of innovation and diversity in its midst.

Firstly, a major benefit has been the more refined creation and modification of modalities and techniques to better meet contemporary needs, for example, the upsurge of brief treatments,²³ experiential, inspirational, cognitive/emotive and other such therapies²⁴ and the application of new technology to therapy, as in biofeedback.²⁵ Secondly, instead of additional division, we

are starting to see the signs of synthesis or at least what Yager²⁶ recently called a Utopian goal—"enlightened eclecticism." Ideally the latter means approaching each clinical situation from multiple theoretical perspectives and selecting those which are in closest accord with an individual patient's needs. Efforts in this behalf are represented by the formulation of integrative models to incorporate dual or multiple, often highly contrasting, schools of thought. Recent examples are the integration of dynamic, behavioral, and experiential schools within the entire array of psychotherapies^{8,27}; the coordination of psychological and somatic treatments, like psychotherapy with pharmacotherapy²⁸; and the development of single eclectic modalities that combine several techniques or approaches under one head, as in Lazarus's multimodal therapy.²⁹

Another aspect of rapprochement is also being conducted on a different level of synthesis, that is, the search for common elements that all therapies share.^{8,30-32} The year 1985 even marked the initiation of the first annual conference of the "Society for the Exploration of Psychotherapy Integration."³³ Of particular interest in this regard has been the current emphasis on the therapeutic relationship in treatment (as a necessary if not sufficient supraordinate influence), evidenced by another "first," a special section devoted to the "therapeutic alliance" in the most recent *Psychiatry Update*.²³ Research along these lines reflects the progress being made in answer to the question not only whether psychotherapy is effective but *how* it heals.

Paradoxically then, the field of psychotherapy is advancing along two ostensibly diametric directions: greater "specificity" in meeting today's needs and tailoring treatments to special populations^{2,3,34} as well as greater "non-specificity" in the amalgamation of diverse approaches and locating generic factors across different modalities.^{8,31,32,35} Indeed, the so-called "specificity/nonspecificity dilemma" is one of the pivotal debates of psychotherapy today, which is being increasingly viewed as a "false contest."³⁶ Strides continue to be made in both areas, as follows.

ISSUES OF "SPECIFICITY"

We overestimate what can be done in short-term therapy and underestimate what can be done in long-term therapy.

One of the major treatment advances has been the development and refinement of a new array of brief therapies and short-term techniques for different patient populations.²³ In keeping with the needs of our time, the advantages of abbreviated treatment modalities are many: reduced financial costs; ability to treat a greater number of patients; direct focus on problems of living, tangible goals, and specific conflict areas; applicability to an extended diagnostic base (e.g., those not amenable to long-term insight-oriented treatment); and the practical virtues of a circumscribed or planned time limit,

which is now even believed to accelerate the curative process.^{37,38} There are also the added attractions, especially for devotees of the analytic tradition, derived from the paucity of proof that brief modalities, which may offer essentially symptomatic relief, are not necessarily superficial, short-lived or subject to symptom substitution. In an era of economic stress and accountability pressure it appears to have become the "therapy qua non" for the criterion of cost-effectiveness.

Although short-term techniques are not exactly new (even Freud's early treatment was brief), what is timely is their application today to an increasing array of symptoms and problems, not only phobias and sexual dysfunctions but presumably larger and more complex syndromes like depressive disorders and borderline personalities. They are also being applied from the vantage points of several theoretical frameworks and their combinations—behavioral, dynamic, cognitive, interpersonal, multimodal. The past, of course, has characteristically associated brief treatment with conditioning and behavior theory with its more typical techniques of systematic desensitization, modeling, assertiveness and social skills training, and self-control procedures. More recent arrivals, however, are encompassing the principles of psychoanalysis, ego psychology, phenomenology, family and social theory, crisis intervention, meditation, and other eclectic Eastern and Western elements to form dynamic, cognitive, and interpersonal brief treatments.

With the advent of psychodynamic approaches to the short-term repertoire,^{23,29} for example, it is now possible to treat a spectrum of patients considered unamenable to more traditional dynamic approaches. Mann and Goldman's⁴⁰ time-limited psychotherapy currently includes a range of suitable candidates that is greater than that of Sifneos's⁴¹ anxiety-provoking therapy or Malan's⁴² later intensive brief psychotherapy. In fact, time-limited formats within a dynamic framework have recently been applied to some of the most refractory diagnostic categories, such as hospitalized patients with borderline personality disorder.⁴³ Instead of having the patient "fit" the already existing approach, and in effect attributing failure of treatment to the personality or characteristics of the *patient* (resistant, nonpsychological-minded, etc.), modifications are now being made in the technique to better accommodate the particular diagnosis or individual.

Not only has the range of suitable or treatable patients been broadened but the prescribed length of treatment is decreasing: In contrast to Sifneos's⁴¹ duration of two months to one year or Malan's⁴² ten to forty sessions, Mann and Goldman⁴⁰ have recently developed a psychoanalytically oriented "interpretive-clarifying-empathic" therapy of only 12 sessions. There have even been reported successes with so-called "single-session psychotherapy."⁴⁴ The crux of such possibilities appears to be the greater focus on patient screening and assessment in relation to treatment planning. Clarkin and Frances,⁴⁵ for instance, have devised a three-step decision tree as a guide to

patient disposition which culminates in the choice between five short-term formats: crisis intervention, psychodynamic, problem-solving, marital-family, or behavioral intervention.

Along with the benefits of new brief therapies has been the delineation of individualized approaches for different patient categories (psychosomatic, geriatric, borderline)³⁴ and efforts to standardize treatment and training so that we can get a better handle on the crucial relationship between practices and outcomes. Noteworthy here is the design of treatments expressly for depressive disorders ("disorder-specific"), which have as their additional unprecedented feature that they are "manual based psychotherapies."⁴⁶ In having long contributed to the operational specification of techniques and goals, such as carefully designated anxiety hierarchies or implosion procedures, behavior therapists may be credited with having prepared the first clinical manuals. Their extension to nonbehavioral formats, which have typically been considered unamenable to specification, represents an important step towards standardization.

In the area of individual treatment of outpatient depression, for example, there are now several manuals.⁴⁷⁻⁴⁹ In particular, two new treatments—cognitive therapy³ and interpersonal therapy² for depression—have been developed with detailed instructions, operational definitions, and case examples for guiding therapists-in-training and for increasing consistency among practitioners. Other manuals have also been devised for different disorders as well as for different therapies, including supportive psychotherapy for nondepressive disorders⁵⁰ and pharmacotherapy for depression.⁵¹

Specific treatments, like cognitive and interpersonal therapy, represent the first steps in refining and empirically testing therapeutic interventions for prescribed populations. That they are highly contrasting approaches reflect the potential application of this type of effort to diverse modalities. (Cognitive therapy, for example, is based on the theoretical premise that neurotic depression is essentially a problem of disturbed thinking and employs both traditional behavioral techniques and cognitive ones in assisting the patient to identify negative views and depressogenic assumptions; by comparison, interpersonal therapy focuses on interpersonal problem areas like role transition and family disputes, utilizing interpretive as well as directive techniques.) Aside from their similarity of diagnostic target population, both share the assets of being explicit about the relationship between theory and technique and having been tested in at least two randomized clinical trials against a pharmacotherapy and a no-treatment control group.⁷

The major benefit of "specificity" is that we can ultimately discover "*What* treatment, for *whom*, is most effective for *this* individual with *that* specific problem, and under *which* set of circumstances?"⁵² To this extent specificity opts for the finest therapeutic tuning in behalf of the individual patient. However, at the present time, the field of psychotherapy is far from

finding the best match of patients with particular methods, types of therapy or styles of therapists. Although the development of treatment manuals for research, training, and treatment can be beneficial in getting us to the above goal through a closer look at what therapists “do” in order to gain greater consistency and consensus of practice, this is both methodologically and psychologically difficult to accomplish. The former difficulty has to do with the complexity of translating complex practices into discrete definable behaviors, while the latter pertains to the risk of loss of spontaneity in the “art” of psychotherapy as the field becomes more technology-oriented, standardized, and “scientific.” Moreover, specificity that produces sectarianism³² is also a mixed blessing: although allegiance to a particular school or belief system may be necessary for the confidence and professional identity of the therapist, it may be antitherapeutic to the patient to the extent that the therapist is unwilling to consider the advantages of alternate approaches.

ISSUES OF “NONSPECIFICITY”

Psychotherapeutic “truth” may reside in the syntheses and integration of divergent schools of thought.

Coupled with the psychotherapy explosion (and perhaps in reaction to it) are the beginnings of a constructive skepticism—that the therapeutic panacea is not to be found in a single school of thought. Recent surveys suggest that over half the professionals interviewed adhere to more than one basic orientation.^{53,54} This means greater appreciation of ingredients in alternate modalities and an increased willingness to incorporate them into one’s own thinking and practice. The current trend away from specificity as “sectarianism” and towards an integrative approach or attitude is gradually manifesting itself in the following ways: (1) transitions and expansions over time *within* individual schools or viewpoints; (2) cooperation and detente *between* different schools or approaches; and (3) unification *beyond* the boundaries of one or more techniques or schools, that is, consensus at a higher level of generalizability.

Transitions and Expansions

Sociological analysis has come to the current conclusion that psychiatry is plagued with a cyclical historical development; the psychiatrist, unlike the indigenous healer, is obliged to “update the symbols of his therapy as culture and society change.”⁵⁵ An overview of the psychotherapies since Freud²⁷ suggests that this evolution of psychological treatment has proceeded from “dynamic” approaches in the 1940s and 1950s, to “behavioral” ones in the 1960s and 1970s, to “experiential” ones in the 1970s and 1980s. Specifically, psychiatry has been marked by the periodic dominance of either “psychological” or “social” or “biological” points of view. Having already wit-

nessed such a progression with both the psychoanalytic and community movements, we are now in the throes of a similar investment and expectation of biological psychiatry as the "new panacea."⁵⁶ Suffice it to say that so varied have been the schools within each model and the nature of their revisions that each has, in fact, virtually "come full circle" in its view of what is therapeutic. Dynamic therapists have become more behavioral (directive, practical, reality-oriented), as behavioral therapies have moved from a conditioning to a cognitive model.⁴⁷

Cooperation and Detente

Detente between different schools or approaches is increasingly seen in the utilization of concurrent treatment regimes, behavioral with dynamic techniques, psychotherapy with sex therapy, and psychotherapy with pharmacotherapy, to name a few.^{8,28,57-60} It is also represented by the forging of new eclectic modalities, like Lazarus's multimodal therapy,²⁹ which combines the physical and the psychological, with techniques ranging from assertiveness training, biofeedback, self-hypnosis, breathing exercises, medication, family therapy, and support groups. In terms of the benefit of integration, a host of efficacy studies of psychotherapy plus pharmacotherapy, for example, reveal an "additive model" that substantiates combined use as superior to either modality utilized alone.²⁸

Unification

Locating integrative principles or generic factors that may transcend individual techniques is a subject of major interest and investigation today.^{8,30,32,61} In fact, 25 years of research by Jerome Frank and his associates have validated the significant roles of expectations, mastery, and emotional arousal as effective ingredients of therapies past and present. Others are exploring and discovering the salience of the therapeutic relationship.⁶²⁻⁶⁴ And yet others have investigated the dynamic curative processes like identification, insight, and working through.⁶⁵ In learning how psychotherapy heals, common factors "can no longer be overlooked," according to most recent research conclusions.⁴

Thus, while "nonspecificity" allows the therapist to transcend his narrow conceptual boundaries and partake of the tremendous diversity of theories and techniques for the potential benefit of the patient, what we do not yet know about combined approaches is probably geometrically proportional to what we have yet to learn about individual techniques. Moreover, unenlightened eclecticism can foster confused therapists with marginal identities. Indeed, Smith, Glass and Miller⁴ have ironically concluded from their meta-analysis of the psychotherapies: "If anything our findings warn against an eclecticism in practice that fails to differentiate into one or another form of psychotherapy. One of the paradoxes of psychotherapy . . . may be that

although all therapies are equally effective, one must choose only one to learn and practice" (p. 185).

DIAGNOSIS

Any classifications, including psychodynamic ones, are necessarily a form of phenomenological reductionism; they deprive both patient and therapist of the immediacy and authenticity of the encounter.

With the specter of DRGs (Diagnostic-Related Groups) before us, the issue of diagnosis for the practice of psychotherapy, and for reimbursement decisions, has become a major concern. The progress that the field of psychotherapy is making in the development and application of diagnostic tools, like advances in treatment, has both benefits and limitations. The arrival of *DSM-III*,¹ a revised classification of psychiatric disorders, has broad implications both pro and con for how the clinician—and the public policymaker—will view the patient, his treatment, and its funding. How *DSM* and other diagnostic criteria will fare in light of impending legislation relates to their relative success as a diagnosis-based treatment guide and as a structure for the multivariate conceptual approach.

On the positive side, *DSM-III* is an improvement over previous instruments in that it utilizes more operationalized and explicit diagnostic criteria, more treatment-oriented classifications, and as its special innovation, allows multiaxial discriminations.^{1,66,67} In particular, its separation of personality disorders (axis II) from all other clinical syndromes (axis I) is seen as a distinct advantage over former classifications.⁶⁷ By highlighting the importance of personality disorders, it ensures that such are not overlooked as attention is directed to the more florid axis I disorders. This is a timely advance in that it is consistent with current evidence that the quantity and quality of preexisting or coexisting personality disturbance may in fact influence the predisposition, manifestations, course, and treatment of various axis I conditions.⁶⁸ As a result, evaluators are now advised to label both the clinical syndrome as well as the personality disorder instead of arbitrarily making a choice between them. This feature, coupled with providing clearly specified diagnostic criteria, is considerably more reliable when compared to prior nomenclatures,⁶⁹ as well as providing better overall reliability for the entire system.

From a more comprehensive viewpoint, others have commented on *DSM-III*'s positive impact on treatment and its generating renewed interest in the subject of diagnosis.^{67,70} It is believed, for example, that the multiaxial system, one of *DSM-III*'s two major innovations, is a significant improvement over previous classifications (*DSM-II* or *ICD*) because the clinician is no longer obliged to compress diagnostic observations into one, all-

encompassing label. Rather, its five-axis option allows for a more conceptually and prognostically complete picture.

At the same time, many critics have raised questions about the limitations of such instruments. Some⁷¹⁻⁷³ have argued that the new system's categories fail to correspond to diagnostic assessment actually needed in clinical practice. Reviewing *DSM-III* from a psychodynamic perspective in terms of its clinical implications, Frances and Cooper,⁶⁷ for example, suggest that it selects material on the psychological surface and limits the clinician's ability to base his diagnoses on subjective responses. One of the more technical clinical questions relates to its definition of stressors; that is, *DSM-III* requires that the therapist rate the stress per se rather than the patient's vulnerability with which the clinician is apt to be more familiar.

There are also other issues that pertain to insufficient information deemed critical for customary clinical practice. One is the inattention to interpersonal diagnosis.⁷² Others feel⁷³ that its evaluation of psychodynamic dimensions are not as clear, precise or reliable as other axes and that it fails to sufficiently deal with a large population of patients for whom psychotherapy is a primary treatment modality, for example, those with affective, anxiety, dissociative, psychosexual, and personality disorders. To compensate for this omission, these authors have recommended the development of a sixth axis to include the patient's conflicts, object relations, coping mechanisms and defenses, and the like.

Other limitations involve the disparity between *DSM-III* definitions and ones recognized in current psychoanalytic practice, such as those for borderline and narcissistic personalities; the nonexistence of the term "neurosis"; possible biological bias in their designation of affective disorders⁶⁷; and the skewing of diagnosis within axis I (e.g., criteria of schizophrenia) which is more appropriate in a medical context than for more global treatment.⁷⁰ There is also the implicit fear of the indiscriminate use of diagnosis-based therapy.

As perhaps a parallel development to that of multivariate diagnoses is the utilization of multivariate statistical techniques, which have aided attempts to validate traditional diagnostic categories and to derive improved ones. For instance, Bartko and colleagues⁷⁴ have devised a flexible diagnostic system for defining schizophrenia, which is derived from several different conceptual and statistical strategies. They have also demonstrated the combined collaboration of clinician and statistician as an integrative approach to assure the maximal collective clinical judgment with systematic data analysis.

In reaction to the overuse of diagnostic instruments, Carpenter and colleagues⁷⁵ have attempted to go "beyond diagnosis" to the phenomenology of psychiatric disorder. Their stance is that the basis of diagnosis, prognosis and treatment should be a comprehensive appreciation of the patient's experiences rather than a few highly specifiable criteria. This viewpoint is

similar to that of Frances and Cooper⁶⁷ in that they believe that descriptive and dynamic orientations in practice overlap and that both are consistent with the current trend towards integration of various schools of thought, in this instance, the collaboration of researcher with clinician.

In summary, the benefits of new diagnostic tools like *DSM-III* include its standardized data base and improved diagnostic reliability for clinical discourse, treatment planning, interpretation of research results, and making funding decisions. In many ways it marks a conceptual breakthrough in diagnosis that is likely to have considerable impact on the future use (and reimbursement) of psychotherapy, especially as DRGs become the basis of health care allocations. Multiaxial diagnoses, despite some crucial flaws, do provide a consensual structure for applying an integrated "bio-psycho-social" model to clinical practice, a definite advance over more myopic classifications of the past.

The limitations of such instruments pertain to their insufficient ability to take into account the human, complex personal qualities and characteristics of patients as individuals. They highlight the gap between the art and science of diagnosis, or between the interests of the clinician and the researcher. Essentially *DSM* is a research-oriented instrument, which requires that there be a minimum of false positive diagnoses in order to obtain a homogeneous sample and to keep any distortion or dilution of statistical data at the lowest possible level. In contrast, clinical practice must insist on a minimum of false negative diagnoses. My own investigation of what clinicians consider as important dimensions in diagnosis and treatment planning revealed more than 50 additional dimensions than those already designated.⁷³ The best that can be hoped is that such limitations can lead to the search for new and more comprehensive criteria in defining disorders, so that neither public policymakers and treatment planners—nor patients themselves—will be shortchanged.

RESEARCH AND REVIEW

The psychotherapist's objective should not be for a Kantian "ultimate" truth, but for Hegelian discursive truths in an ongoing search for partial answers.

In a 40-year appraisal of psychotherapy (1940–1980) it was concluded that "The period of accountability is clearly upon us."⁷⁶ Garfield⁷⁷ considers the renewed concern about psychotherapy effectiveness a consequence of three currently converging factors: "(1) the matter of third-party payments; (2) the swing from a psychoanalytic to a more biological emphasis in psychiatry; and (3) criticism of existing research on outcome by behaviorists" (p. 295). Although it has been nearly 35 years since Eysenck's⁷⁸ controversial challenge of psychotherapy's efficacy, the field is still struggling with this

question, and it is only recently that significant strides are being made to counter his conclusions.^{4,5,12,62,79,80}

Reviews of research in psychotherapy reveal not only enormous growth in the number, range, and innovativeness of studies of therapeutic efficacy, but most important, in the nature of controlled investigation. These improvements have occurred in more refined definitions of patient populations, applying new standardized diagnostic criteria and psychometric tests; greater specification of treatment techniques, utilizing written manuals, audio- and video-tapes for establishing consensual guidelines of practice; operationalizing diverse and complex outcomes, using a range of measures by patient, family, therapist and other observers; viewing psychotherapy as an "emergent phenomenon" adding interactional and process variables that assess the course of therapy at various points in time; and tackling the perennial problem of control groups, with comparative data derived from recipients of different types of treatment, untreated patients, and those on waiting lists.⁵

As a result of such studies, various therapeutic myths and self-protective past presumptions are being dispelled.⁸¹ We are beginning to acknowledge, for example, that not one but a wide range of therapies may be equally effective; that therapist experience is not necessarily a direct determinant of therapeutic success; that therapeutic effects are not always positive or benign; that most therapies have their greatest potency early in treatment and that where there may be initial differences in effectiveness these usually disappear in time. Indeed, that classic conclusion of the 1970s with regard to comparative outcome⁷⁹—that "everyone has won and all must have prizes," has essentially been confirmed and supplanted in importance by the newly conceived efficacy studies of Smith, Glass and Miller,⁴ today's answer to Eysenck.

The aforementioned studies are credited with being the most extensive in a series of attempts to organize and synthesize the results of prior investigations.^{79,82,83} In contrast to Luborsky and associates' "box-score" method of interpreting research conclusions (which used 12 criteria of scientific quality) on 33 studies, Smith and colleagues aggregated a much more sweeping array (475 studies) using a "meta-analysis," which first coded the studies on a set of outcome-related variables, then quantified and correlated them with outcome. On the positive side, this innovative statistical procedure marks perhaps the most intriguing and provocative research analysis to date. It is unique on several counts: it incorporates an enormous spectrum of varied formats; is extremely comprehensive, encompassing several hundred studies in its first major application; estimates not just treatment effects but their actual size or magnitude; and examines complex correlations between clients, therapists, techniques, and treatment effects. In brief, its "evidence overwhelmingly supports the efficacy of psychotherapy," with an average "effect size" (ES) of .85 units in favor of treatment patients; this means that the

average person in psychotherapy was found to be better off at the end of treatment than were 80 percent of control subjects. In fact, this ES may actually have been an underestimate because the authors included many influences under the rubric of treatment (e.g., counseling, even placebo). When these were excluded, the ES was .93—higher than the figure estimated for coronary bypass in the medical care of angina (i.e., .80).

Despite the advances of such findings, these studies are not beyond criticism. Although their results are considered replicable and reliable, validity is harder to establish. We realize that therapies, patients, objectives, and formats of treatment were diverse, and that the samples were generally small (average of 30 patients). Despite the fact that the authors vouch for the validity based on their aggregate population of 25,000, it is still questioned whether many, sometimes poorly controlled studies, are comparable to a single large valid study.

In this and in other investigations, the ambiguities of conclusions are based on the limitations of the scientific method in clinical research. Can the rigors of biological research, for instance, be applicable to the study of psychological interventions, which may represent a human relationship more than an operational technology? Attempting to answer the deceptively simple question, "Is therapy effective?" must take into account the diversity of theoretical and clinical approaches to psychotherapy, the difficulties in describing and measuring it as a uniform practice, the range of disorders, the variety of settings, the large number of patient, therapist, and interactional variables—seemingly *ad infinitum*. Furthermore, therapists (often researcher versus clinician) have political, economic, and narcissistic interests that divide them not only along professional identities or roles, but along ideological lines. Therefore, biases may operate on many, often insidious levels both methodologically and interpretively.

To the topic of the benefits and limitations of research and review must also be considered not only the parts played by clinician and researcher but other concerned parties, including the psychotherapist's patients, his professional peers, and most pertinent today, the public policymakers. The former groups' concerns are centered on the issue of ethics, the latter on economics.

As to ethical issues, the timely theme of Stone's 1980 Presidential Address of the American Psychiatric Association⁸⁴ was "conceptual ambiguity and morality in modern psychiatry." Although he fantasizes about a new era called "pragmatic eclecticism" in which four competing models (biological, psychodynamic, behavioral, social) peacefully coexist, his concern is that having overcome the tyranny of narrow orthodoxies, therapists are now in danger of retreating behind new expedient walls that discourage "the courage of conscience." He applies this to at least three special areas with which the field has been recently grappling: racism, sexuality, and women's issues.

For these as well as other areas the lack of a viable code of ethics for the

psychotherapies is one of the crucial constraints of the 1980s.^{10,85} Although an instrument of this order has been formulated⁸⁶ it is believed that such codes are of limited utility because they cannot really assist the individual psychotherapist in his daily practice. They are also disappointing to those who may confuse codes with covenants and expect to magically produce morally scrupulous practitioners. Zitrin and Klein,⁸⁷ for example, insist that the more pressing problem is not the establishment of guidelines but their enforcement; we cannot rely on peer review which is fraught with personal conflict, fear of liability, inaccuracy, and bureaucratic barriers.

At best it is believed that ethical problems in psychotherapy remain unresolved by definition; they are necessarily part of an ongoing search. Some recommendations in this behalf are greater exploration of the ethical assumptions and presumptions on which psychotherapy is predicated; openness to consultation with others towards making the best consensual ethical decisions; and constant examination of one's own attitudes and behaviors within and outside the therapeutic relationship.¹⁰

The subject of psychotherapy review is not only an ethical problem but more practically, an economic one, as federal bodies are making increasing decisions about allocation of resources for health and mental health care. This is having important implications for five activities which currently involve governmental responsibility: accreditation of facilities, licensing of professionals, judgments as to safety and efficacy of treatment, use of public monies for funding and reimbursement, and lastly, research.⁸⁸

As to the latter, Kiesler⁸⁹ points out that research findings and discussions of psychotherapy effectiveness, with which the field is currently preoccupied, have to be placed in the context of overall mental health policies, existing systems of care, population needs, and treatment alternatives. In the final analysis, the benefits and limitations of psychotherapy vis-a-vis cost effectiveness criteria depend on whether one takes a "bottom-up" or a "top-down" approach. For example, he demonstrates that with a "bottom-up" analysis, which is the more traditional way of looking at mental health policy, there has been continued good progress in enlarging the number of service providers available, enhancing the training of mental health professionals, etc. But from the "top-down," psychotherapy as typically practiced is too labor intensive to be a viable solution to overall national psychiatric problems (i.e., he estimates that only 2 percent of those needing services could be treated).

Taking a rather paradoxical position on the matter, Schlesinger and Mumford⁹⁰ feel that public policy is not dependent or limited by the lack of conclusive evidence of the effectiveness of psychotherapy, but that political forces rather than evidence may well determine its ultimate place in our health care delivery system. By summarizing reviews of psychiatric and medical morbidity surveys, psychotherapy outcome study literature, and

cost-offset studies of patients in health maintenance organizations and in fee-for-services practices, they demonstrate

ample evidence that (1) psychotherapy in general is effective as a treatment for conditions for which it is usually undertaken. . . . (2) Psychotherapy significantly affects physiological and medical indicators. When used adjunctively for patients with acute or chronic physical diseases, recovery is speeded and costs are reduced . . . [and] (3) Psychotherapy can also be cost-effective by reducing inappropriate and possibly dangerous use of medical services.

They conclude that "it is clearly not a matter of how to formulate public policy in the absence of evidence. Rather we should question why the ample evidence that is available is not used for the formation of public policy" (p. 386).

In sum, the potential benefits of continued research and review are inestimable; the limitations will depend on methodological and interpretive problems as well, of course, as on who shall be the recipients of the information and how it is ultimately utilized.

CONCLUSION

The 1980s offer psychotherapy a sense of proportion, and thus are thoroughly anti-romantic.

A look at the psychotherapies of today suggests that we are currently dealing in unparalleled proportion with both benefits and limitations. The former is manifested in continued investigation and progress in treatment, training, diagnosis, research, and review. A special challenge confronts the modern multifaceted therapist who is increasingly called upon to be eclectic in knowledge and holistic in practice, especially as we learn more about the nonspecific as well as specific aspects of psychotherapy. Moreover, an "age of ethical crises" is being superseded by an era of economics in which the present psychotherapist is the target of at least triple review (outside of himself)—by not only his patients and professional peers but, more than ever before, public policymakers.

However, in light of the practical pressures and progress in the 1980s that are providing the field with a stringent sense of proportion and moving us closer to a "technological model of psychiatry,"¹³ it is necessary to question the limitations of this process. After years of training "practitioners of techniques" there is neglect of the "art of midwifery" in psychotherapy; by laboriously classifying and categorizing patients, less attention is being paid to the person behind the diagnosis. In thereby stressing the science of psychotherapy and emphasizing therapeutic efficiency, we may be in danger of "shortchanging" the spontaneity of the therapist as well as the humanity of the patient, which no operational variable or economic calculation can sufficiently recognize. For example, how does one compute the patient's

ontological hunger, or his yearnings for dependency, intimacy and infinity? And how does one measure the essence of the clinician, who needs to remain open and flexible in order to fully appreciate the complexity and diversity of patients as individuals? Thus, ironically, as psychotherapy in this decade becomes more "cost-effective," we still have to ask. . . . At what price?

SUMMARY

The promises and challenges of the psychotherapies today are explored in the areas of treatment, training, diagnosis, research, and review. Benefits discussed include advances in the science and standardization of psychotherapy assessment and practice; improved research tools for establishing therapeutic efficacy; development of new clinical modalities, especially short-term techniques; cooperative efforts to combine and integrate diverse methods of treatment; establishment of ethical codes of conduct for the therapist; and the application of computerized evaluation and cost-containment procedures. At the same time, limitations reflect the various conceptual confusions, research inadequacies, identity crises, political pressures, and economic constraints of the field. Of particular concern is the current "age of accountability" which has placed cost-effectiveness at the pivot of public policy in mental health care.

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